Public health advice on Bundibugyo virus disease for internally displaced people, refugees and migrants living in camps or camp-like settings

Overview

Bundibugyo virus disease (BVD) is a severe and often fatal form of Ebola disease, caused by the Bundibugyo virus (BDBV), a species of *Orthoebolavirus*. Early detection, isolation and care, infection prevention and control, and community engagement are critical to reducing transmission and saving lives, particularly in crowded settings where outbreaks can spread rapidly.

Authorities, humanitarian organizations, service providers and outbreak responders working with internally displaced people, refugees and migrants in camps and camp-like settings play a key role in preventing transmission, identifying suspected cases early and ensuring timely access to health, protection and support services. Responses should be coordinated, practical and adapted to the realities of overcrowded and resource-constrained environments *(1, 2)*.

All actions for these populations should be guided by principles of equity, dignity, community engagement, accountability and "do no harm" *(3)*. People should be able to access services without discrimination, delays or documentation barriers, while frontline workers must be adequately protected and supported.

Meaningful engagement with people living in camps or camp-like settings, as well as with host communities, is essential for identifying acceptable and workable solutions, understanding local concerns and establishing effective feedback mechanisms.

This public health advice provides practical steps for authorities and partners to help prevent and control the spread of BVD and ensure access to appropriate protection measures for people living in camps and camp-like settings. It is based on current evidence and good public health practices *(4–6)* and is intended to complement guidance from national health authorities and humanitarian partners. This public health advice should be adapted to the local context in consultation with camp authorities, displaced populations and host communities.

Purpose

This public health advice is for:

- relevant government departments, such as those responsible for health, social care, housing, immigration, mental health and child poverty;
- national, subnational and local authorities;
- facility managers, service providers, camp managers and relevant authorities;
- humanitarian organizations, faith networks and civil society organizations;
- health-care professionals and all categories of health-care workers;
- community leaders and community health and social workers; and
- responders involved in the BVD outbreak emergency response.

Adapting public health advice to local settings



Public health advice should always reflect the realities, priorities and capacities of the communities where it is applied *(1, 2)*. People living in camps and displacement settings may face challenges that make some recommended public health measures difficult to implement. Households may share limited living space, multiple family members may sleep in the same room, population movements may be frequent and access to health, water, sanitation and hygiene services may be constrained *(7)*.

Public health advice should therefore be adapted to local circumstances and developed together with affected and host communities. Community engagement is essential to identify practical, locally acceptable and culturally appropriate solutions that reduce risks while protecting dignity, social cohesion and access to essential services.

When adapting public health advice, authorities and partners should consider the following circumstances.

Context and living conditions

- local living conditions and available services, according to Sphere standards *(8)*
- security and access constraints
- patterns of population movement within and between camps, host communities and across borders
- how to ensure and promote hand hygiene facilities in all social gathering places, near toilets, near water points, food distribution points, etc.
- availability of referral systems and health systems.

Community perspectives and communication

- community priorities and concerns
- languages, dialects, literacy levels and preferred communication channels
- cultural, religious and social practices
- continuous community feedback, complaints and accountability mechanisms.

Equity, protection and inclusion

- equitable access to health, protection and essential services without discrimination, regardless of legal status
- barriers to accessing services, including fear, stigma, arrest, detention or deportation
- gender, age and disability considerations
- needs of groups at higher risk of severe disease, exclusion or protection concerns
- risks of social exclusion, family separation, livelihood disruption and interruption of essential services.

Safety and safeguarding

- prevention of sexual exploitation, abuse and harassment
- protection of privacy, confidentiality and dignity
- measures to ensure outbreak response activities do not create additional harm.

Preparedness and anticipatory action

- existing community capacities and coping mechanisms
- opportunities to strengthen preparedness, readiness and early action before risks escalate.

Adaptation should seek not only to reduce disease transmission, but also minimize the social, economic and protection impacts of the outbreak on affected populations.

Key context-specific advice for camps and camp-like settings during BVD outbreaks



1. Prevent transmission

Early recognition of symptoms and prompt treatment in camps and camp-like settings are essential to improve survival and reduce transmission, where overcrowding and shared facilities increase the risk of spread. Authorities and partners should ensure that people living in camps and camp-like settings receive timely, accurate and easy-to-understand information about the disease through trusted channels *(4)*. Community members, outreach workers and camp leaders should be equipped to support early detection, timely referral, and access to testing and treatment, which helps to reduce transmission and protects the health of camp residents *(9)*.

Key messages for communities in these settings include the following.

- Seek medical care immediately if you develop symptoms such as sudden fever, muscle pain, fatigue, headache, abdominal pain, lack of appetite, sore throat, nausea, vomiting or diarrhoea.
- Avoid contact with the blood, vomit, diarrhoea or other body fluids of people who are sick.
- Avoid contact with contaminated bedding, clothing and personal belongings.
- Know how to report illness and how to access referral, testing and treatment services, including how to contact camp health services (for example, hotline numbers or text message codes).
- Report suspected cases, unusual illness clusters and unexplained deaths through established community reporting mechanisms.
- Immediately notify the BVD alert system of any suspected BVD case, and/or refer the patient.
- Cooperate with public health measures, including contact tracing and health monitoring.

If someone becomes sick at home, do the following:

- Notify the designated focal point or call the alert number immediately.
- Avoid direct contact with body fluids.
- Use available barriers (such as gloves or plastic sheeting) if providing care only while waiting for the patient to be referred to a health centre.
- Wash hands thoroughly and frequently, particularly after any contact with the sick person.
- Keep the sick person separated from others, where feasible, until a medical assessment is available.

Practical measures should be identified and developed with displaced and host communities, camp management and health authorities.

2. Ensure access to health services

Early access to treatment and supportive care is essential to improve survival and reduce transmission. Access to health services should be equitable, inclusive and free from discrimination. People living in camps and camp-like settings may face barriers such as distance, lack of transport, fear of costs or uncertainty about where to seek care *(9)*.

Authorities and partners should do the following.

- Establish clear referral pathways between shelters, camp posts, community health services, clinics, mobile teams and hospitals.
- Provide transport and patient accompaniment where required and communication support to facilitate timely access to care.
- Ensure communities know where and how to access testing, treatment and referral services, and encourage them to seek care immediately if they develop symptoms.
- Remove administrative and documentation requirements that may delay access to care, and ensure services are accessible, regardless

of nationality, migration status, refugee status, displacement status or documentation.
- Maintain continuity of health services, including protection, psychosocial, maternal and reproductive health, disability support and food assistance.
- Ensure public health measures do not prevent people from accessing essential assistance or communicating with family members.

Treatment centres and health facilities should provide care that respects dignity, privacy and cultural considerations.

3. Infection prevention and control measures

Effective infection prevention and control are essential to preventing and reducing BVD transmission, particularly in camp settings where living space may be limited and sanitation facilities are often shared. Measures should reduce exposure in homes, health facilities, latrines, isolation areas and communal spaces *(10)*.

Authorities and partners should do the following.

- Ensure hand hygiene stations are available at key locations.
- Provide hand hygiene supplies, including soap, and information on their correct use.
- Train camp residents and community volunteers to identify BVD signs and symptoms and, when detected, to immediately seek medical support and notify camp authorities.
- Support households in order to limit members' contact with sick individuals, and promptly notify health authorities when symptoms develop.
- Encourage separation of sick individuals from other household members, where feasible, while awaiting medical assessment.
- Promote safe practices, including avoiding the sharing of potentially contaminated personal items such as clothing, bedding, towels and eating utensils.
- Ensure communities understand and follow guidance provided by health workers on immediate isolation, the referral system and safer management of sick individuals while waiting for referral.
- Discourage families from transporting sick individuals (suspected BVD sick person), as transportation must be done by health authorities.

4. Guarantee water, sanitation and hygiene

Access to safe water, sanitation and hygiene services is especially important in camp-like settings where people often share water points, toilets, bathing facilities, social gathering places and waste systems. Maintaining these services according to Sphere standards helps reduce disease transmission and protect community health *(8)*.

Authorities and partners should do the following.

- Ensure access to sufficient safe drinking water and functional handwashing with soap facilities.
- Keep latrines, bathing areas and waste systems clean and usable.
- Improve environmental cleaning in common spaces and high-use facilities such as social gathering places and laundry areas.
- Regularly distribute soap and personal hygiene supplies.
- Prioritize densely populated blocks, underserved locations and newly arrived populations.

Communities should be supported to do the following.

- Use safe water sources and store drinking water in clean, covered containers.
- Follow recommended household water treatment methods, such as chlorination or other methods advised by authorities.
- Use designated toilets or latrines and practice safe disposal of child faeces and household waste.
- Wash hands regularly with soap and water, particularly after using the toilet and before preparing or eating food.
- Keep shelters, kitchens, cooking areas, toilets, washing areas and communal areas clean.
- Report damaged or overflowing sanitation facilities to camp management or service providers.
- Follow instructions from health, safe water, sanitation, hygiene and camp management teams on hygiene, environmental cleaning and waste management.

5. Proactively communicate, engage and support communities

Camp populations may have limited trust in formal systems, particularly if they have experienced displacement, violence or repeated service disruptions. Risk communication and community engagement are essential to building trust, supporting participation and enabling effective outbreak response *(3, 4, 7, 11)*.

Authorities and partners should do the following.

- Establish accessible feedback, complaints and accountability mechanisms for listening, and integrate the community concerns and priorities in the response.
- Provide timely, accurate and easy-to-understand information through trusted and accessible communication channels.
- Codevelop culturally appropriate messages and solutions with affected communities.
- Engage and integrate trusted community leaders and networks to encourage participation, peer support and community-led action.
- Engage survivors, where feasible and accepted by survivors, as trusted advocates and peer supporters.
- Identify, monitor and address rumours, misinformation and community concerns.
- Promote solidarity and social cohesion between displaced and host communities.
- Ensure people moving within or between camps, communities and borders know where and how to access care if symptoms develop.

6. Encourage safe and dignified burials

Communities play a central role in preventing BVD transmission during burials. Compassionate dialogue can help families understand the need for safe burial measures, while also enabling them to participate in culturally and religiously appropriate mourning practices. Community engagement and trust are essential to ensuring that safe and dignified burials are accepted and implemented *(12)*.

Authorities, partners and community structures should do the following.

- Encourage prompt reporting of deaths to enable safe management.
- Ensure trained burial teams are available and are able to respond rapidly.
- Involve families and faith and community leaders in adapting safe and culturally respectful rites, while clearly explaining transmission risks.
- Prevent unsafe washing, touching, kissing or any other contact with the body.
- Train burial teams to integrate cultural sensitivity, privacy and dignity into safety procedures.
- Promote community participation in safe and dignified burial activities, and consider including trained female staff where appropriate.

7. Protect vulnerable populations and ensure equitable access to assistance

Camp and camp-like settings often include populations who may face increased risks of severe disease, barriers to care or exclusion from services *(11)*.

Authorities and partners should identify and address the specific needs of the following people:

- pregnant, breastfeeding women
- children and adolescents
- older people
- malnourished individuals
- people living with HIV, immunocompromised individuals and those with chronic health conditions
- people with disabilities
- survivors of violence and other vulnerable populations.

Authorities and partners should do the following.

- Use targeted outreach and provide additional support for high-risk households.
- Support residents to recognize symptoms, notify health authorities and seek care promptly.

- Ensure services, assistance and information are accessible, inclusive and adapted to different languages, literacy levels and disability needs.
- Recognize that caregiving responsibilities may increase exposure risk, particularly for women and girls.
- Address risks of gender-based violence associated with isolation or movement restrictions.
- Ensure vulnerable groups can continue to access essential services and support throughout the outbreak.

Protection considerations should be integrated into all outbreak preparedness and response activities to ensure safe and confidential access to protection and support services.

8. Protect health-care workers and frontline responders

Health-care workers, community health workers, volunteers and outbreak responders may face repeated exposure, fatigue and security concerns. Protecting them is essential to sustaining the outbreak response and maintaining essential health services *(13, 14)*.

Health authorities and partners should do the following.

- Provide appropriate training, supervision and practical guidance on infection prevention and control measures.
- Ensure the availability and correct use of personal protective equipment.
- Support implementation of infection prevention and control procedures in health facilities and community settings.
- Provide occupational health and psychosocial support services.
- Promote community support and acceptance of health workers and responders.
- Protecting health workers and caregivers helps ensure the continuity of essential health and community services throughout an outbreak.

9. Combat stigma, discrimination and exclusion

Stigma, discrimination and exclusion can undermine outbreak control by discouraging people from seeking care, accessing services and cooperating with public health measures. In camp settings, these effects can be especially harmful because people rely on shared services and strong social networks *(9)*.

Authorities, partners and community leaders should do the following.

- Protect the dignity, privacy and confidentiality of affected individuals and families.
- Address discrimination against refugees, migrants, internally displaced people, survivors and other affected populations.
- Avoid language that blames affected people or communities.
- Promote social inclusion, solidarity and peaceful coexistence between displaced and host communities.
- Support the safe reintegration of survivors into communities.

An inclusive and rights-based approach is essential to building trust and supporting effective outbreak response.

10. Provide mental health and psychosocial support

People living in camps and camp-like settings may experience significant psychosocial distress during a BVD outbreak. Mental health and psychosocial support is an essential component of the outbreak response and should be accessible to affected people, including displaced persons *(15)*.

Authorities, partners and community leaders should do the following.

- Ensure mental health and psychosocial support is available throughout the BVD care continuum, including for people awaiting results, in isolation and following discharge.
- Train health and social-care workers, community health workers, volunteers and frontline responders to provide psychological first aid.
- Promote awareness of common stress reactions to outbreaks, such as fear, grief and sleep disturbance, and encourage help-seeking.

- Ensure continuity of mental health services throughout the outbreak, including access to psychotropic medication.
- Maintain access to mental health services and establish referral pathways from community-level psychosocial support to primary health care and specialist mental health services.
- Ensure unaccompanied children and children who have lost family members receive psychosocial support and are linked to child protection services.
- Ensure health-care workers, volunteers and frontline responders have access to mental health and psychosocial support, including psychological first aid, peer support and, where needed, mental health care.

11. Protect against sexual exploitation, abuse and harassment by aid workers

During outbreaks and emergencies, people may depend heavily on responders for health care, shelter, food, water, transport and other essential services. These situations create power imbalances and increase the risk of sexual exploitation, abuse and harassment, particularly for women, girls, boys, people with disabilities and other vulnerable groups. Isolation, fear of stigma and reduced access to services can further heighten these risks *(16)*.

Authorities and partners should do the following.

- Ensure communities know how to safely report concerns and seek support.
- Communicate clearly, through a variety of trusted channels, that assistance is as follows:
    - free, and no exchange for benefits is tolerated
    - provided with dignity, respect and confidentiality
    - accessible regardless of nationality, refugee/displacement status, age, sex, disability, religion or ethnicity.
- Establish accessible, confidential and survivor-centred reporting and complaints mechanisms.
- Ensure all personnel, volunteers and contractors comply with codes of conduct and receive safeguarding training.
- Implement practical prevention measures (for example, appropriate staffing and privacy for intimate support; supervision of field staff; and safe processes for registration and service delivery).
- Respond promptly to allegations through appropriate safeguarding and accountability procedures.
- Ensure availability and accessibility of referral pathways for protection, gender-based violence/sexual violence services and other survivor support.

BVD – key facts

- BVD spreads through direct contact with the blood or other body fluids of a person who is sick or who has died from the disease, or through contact with contaminated objects and surfaces. Transmission can also occur during unsafe burial practices.
- The illness usually begins with the sudden onset of fever with nonspecific symptoms, including fatigue, muscle pain, headache, sore throat, abdominal pain, loss of appetite, nausea, vomiting, diarrhoea and, in some cases, rash.
- Symptoms usually appear between 2 and 21 days after exposure. People are not considered infectious until they develop symptoms.
- There is currently no licensed vaccine against BDBV. The approved Ebola vaccine currently works for Ebola virus disease, which is caused by a different virus species.
- There are currently no licensed therapeutics for BVD. Candidate therapeutics are available for evaluation during outbreaks, but there is currently no evidence of their effectiveness against BDBV. Clinical trials will help assess their safety and benefits. However, early and supportive medical care significantly increases the chances of survival.
- Early medical care saves lives. Anyone with symptoms should seek medical attention immediately as early, supportive treatment significantly improves the chances of survival.
- Immediately notify the BVD alert system of any suspected BVD case, and/or refer the patient *(5)*.

Methodology

This public health advice was developed by consolidating operational and field observations from previous and ongoing Ebola responses in the Democratic Republic of the Congo, together with the published technical literature relating to similar filoviruses and the management of disease outbreaks in camp and camp-like settings in the relevant technical areas. The document had inputs and several rounds of review by technical experts and members of the World Health Organization (WHO) incident management team and partner organizations to ensure that the content aligns with current best practices and takes into account relevant experience from Ebola outbreak responses.

Acknowledgements

WHO gratefully acknowledges the many individuals who contributed to the development of this public health advice.

This document was prepared by Thomas Moran and Vicky Houssiere under overall leadership of Stella Chungong and Kai von Harbou from the WHO Health Emergencies Programme at WHO headquarters.

Special thanks are due to the following individuals who provided technical input and reviewed the document: Lucas Alamprese (World Food Programme, Italy), Anastasiia Atif and Aarunima Bhatnagar (United Nations Children's Fund, Kenya), Elisabetta Brumat (United Nations High Commission for Refugees, Switzerland), Thiaba Fame (International Federation of the Red Cross and Red Crescent Societies, Senegal), Lasha Goguadze (International Federation of the Red Cross and Red Crescent Societies, Switzerland), Yoko Iwasa, Allen Maina and Maheswari P Murugayia (United Nations High Commission for Refugees, Switzerland), Asaad Kadhum (United Nations High Commission for Refugees, Jordan), Carolyn Kipsang and Kit Leung (International Organization for Migration, Switzerland), Paidamoyo Magaya (Africa Centres for Disease Control, Ethiopia), Yves Stephane Ngaleu (International Federation of the Red Cross and Red Crescent Societies, Kenya), Dorothy Wambeti Njagi (International Organization for Migration, Switzerland), Michael Smith (World Food Programme, Italy), Elizabeth Haley West and Abdoulaye Wone (International Organization for Migration, Switzerland).

Special thanks are also due to the following individuals from the WHO Health Emergencies Programme, WHO headquarters, who provided technical inputs: April Baller, Mahmoud Hamouda, Anaïs Legand, Eba Pasha, Jamie Rylance, Nosheen Usman and Niluka Wijekoon Kannangarage.

Declaration of interests

All external experts submitted to WHO a declaration of interest disclosing potential conflicts of interest that might affect, or might reasonably be perceived to affect, their objectivity and independence in relation to the subject matter of this guidance. WHO reviewed each of the declarations and concluded that none could give rise to a potential or reasonably perceived conflict of interest related to the subjects covered by the public health advice.

Resources

1. Camp management toolkit. Geneva: International Organization for Migration, Norwegian Refugee Council and United Nations High Commissioner for Refugees; 2015 (https://www.cccmcluster.org/resources/camp-management-toolkit).

2. Scaling up COVID-19 outbreak readiness and response operations in camps and camp-like settings: interim guidance. Geneva: International Federation of Red Cross and Red Crescent Societies, International Organization for Migration, United Nations High Commissioner for Refugees and World Health Organization; 2020 (https://interagencystandingcommittee.org/camp-coordination-and-management/interim-guidance-scaling-covid-19-outbreak-readiness-and-response-operations-camps-and-camp-settings).

3. Community engagement and accountability (CEA) toolkit. Geneva: International Federation of Red Cross and Red Crescent Societies; 2021 (https://www.ifrc.org/document/cea-toolkit).

4. Risk communication and community engagement readiness and response toolkit: Ebola disease. Geneva: World Health Organization; 2025 (https://iris.who.int/handle/10665/381929). Licence: CC BY-NC-SA 3.0 IGO.

5. WHO fact sheet: Ebola disease [website]. Geneva: World Health Organization (https://www.who.int/news-room/fact-sheets/detail/ebola-virus-disease, accessed 20 July 2026).

6. Ebola and Marburg virus disease epidemics: preparedness, alert, control, and evaluation, interim version 1.2. Geneva: World Health Organization; 2014 (https://iris.who.int/handle/10665/130160).

7. Defining community protection: a core concept for strengthening the global architecture for health emergency preparedness, response and resilience. Geneva: World Health Organization; 2024 (https://iris.who.int/handle/10665/379055). Licence: CC BY-NC-SA 3.0 IGO.

8. The Sphere handbook. Humanitarian charter and minimum standards in humanitarian response. Geneva: Sphere Association; 2018 (https://spherestandards.org/handbook-2018/).

9. UNHCR Emergency Handbook [website]. Geneva: United Nations High Commissioner for Refugees (https://emergency.unhcr.org, accessed 20 July 2026).

10. Infection prevention and control guidelines for Ebola and Marburg disease. Geneva: World Health Organization; 2026 (https://iris.who.int/handle/10665/385879). Licence: CC BY-NC-SA 3.0 IGO.

11. Minimum quality standards and indicators for community engagement. New York: United Nations Children's Fund; 2020 (https://www.unicef.org/mena/reports/community-engagement-standards).

12. Field situation: How to conduct safe and dignified burial of a patient who has died from suspected or confirmed Ebola virus disease. Geneva: World Health Organization; 2014 (https://iris.who.int/handle/10665/137379).

13. Clinical management of patients with viral haemorrhagic fever: a pocket guide for front-line health workers: interim emergency guidance for country adaptation. Geneva: World Health Organization; 2016 (https://iris.who.int/handle/10665/205570).

14. Epidemic control for volunteers: training manual. Geneva: International Federation of Red Cross and Red Crescent Societies; 2021 (https://www.ifrc.org/document/epidemic-control-volunteers-training-manual).

15. Psychological first aid during Ebola virus disease outbreaks, 2nd ed. Geneva: World Health Organization; 2024 (https://iris.who.int/handle/10665/379661). Licence: CC BY-NC-SA 3.0 IGO.

16. Protection from sexual exploitation and abuse and sexual harassment unit [website]. Geneva: International Organization for Migration (https://www.iom.int/protection-sexual-exploitation-and-abuse-and-sexual-harassment, accessed 20 July 2026).

© **World Health Organization 2026**. Some rights reserved. This work is available under the CC BY-NC-SA 3.0 IGO licence.

Suggested citation. Public health advice on Bundibugyo virus disease for internally displaced people, refugees and migrants living in camps or camp-like settings. Geneva: World Health Organization; 2026. https://doi.org/10.2471/B09887